Woodward Academy Model UN
Background Guide
Historical Crisis Committee
Spanish Flu

In 1918, the world was already dealing with World War I when another major crisis began to spread. A serious outbreak of influenza moved through military camps, cities, and eventually countries across the world. One of the first major recorded outbreaks in the United States occurred in March 1918 at Camp Funston in Fort Riley, Kansas. More than 100 soldiers became sick, and within one week the number of cases had increased five times over.[1]The illness became known as the Spanish Flu, although there was no proof that it began in Spain. Spain was neutral during World War I, so its newspapers were able to report openly on the outbreak while many countries involved in the war limited information that could hurt public morale. This caused the disease to become strongly associated with Spain even though its true origin was uncertain. The first wave during the spring of 1918 was relatively mild, but a much more dangerous second wave appeared in the fall. This wave was especially concerning because many young and otherwise healthy adults were becoming seriously sick or dying, which was unusual for influenza.[2] By October 1918, hospitals were becoming overcrowded, medical workers were overwhelmed, and governments were struggling to control the outbreak. In the United States alone, an estimated 195,000 people died from influenza during October.[3]

World War I and the Spread of Influenza

World War I played a major role in how quickly influenza spread. The war began in 1914 and involved countries including Britain, France, Russia, Germany, Austria-Hungary, and eventually the United State. By 1918, millions of soldiers were living and traveling in crowded conditions. After the United States entered the war in 1917, large military camps were created that could hold between 25,000 and 55,000 soldiers at a time. By May 1918, hundreds of thousands of American soldiers were being transported across the Atlantic each month. Soldiers often lived in crowded barracks, traveled on troop ships and trains, and spent time in military hospitals. These conditions made it much easier for influenza to move from person to person and then spread to new areas. The CDC identifies overcrowding and worldwide troop movements as important reasons the disease spread so rapidly.[4] A soldier could become infected at a military base and

[1] Centers for Disease Control and Prevention. “1918 Pandemic Influenza Historic Timeline.” *Pandemic Influenza.*
[2] National Archives and Records Administration. “The Influenza Epidemic of 1918.
[3] Centers for Disease Control and Prevention. “History of 1918 Flu Pandemic.
[4] Centers for Disease Control and Prevention. “1918 Pandemic Influenza Historic Timeline.

then carry the illness to another city or country before officials fully understood what was happening. This created a difficult decision for military leaders. By the fall of 1918, major battles were still taking place in Europe. Limiting troop movements could help slow the spread of influenza, but it could also weaken a country's position in the war. Continuing to move soldiers, however, could cause even more outbreaks.

What Scientists Knew in 1918

Delegates must remember that medical knowledge in 1918 was very different from what is known today. Scientists understood the basic idea of germ theory, which explained that microorganisms could cause disease. The work of scientists such as Louis Pasteur and Robert Koch had changed how doctors understood infections and encouraged researchers to search for the specific organism causing influenza.[5] However, scientists did not yet know that influenza was caused by a virus. Many researchers believed that a bacterium called Pfeiffer's bacillus caused the disease. The bacterium had been discovered in the 1890s and was found in some influenza patients. By 1918, scientists were beginning to question this idea because the bacterium did not appear in every person who had influenza. This lack of knowledge made treatment extremely difficult. There were no modern antiviral medications, and antibiotics did not yet exist. Many influenza patients developed pneumonia after becoming sick, and doctors had very few effective ways to treat these infections. Because there was no reliable cure, governments had to focus heavily on preventing the disease from spreading. Scientists also attempted to create vaccines. Because many believed Pfeiffer's bacillus caused influenza, experimental vaccines were developed using the bacterium. William H. Park of the New York City Health Department worked on one such vaccine in 1918, while researchers at institutions including Tufts Medical School and the University of Pittsburgh developed others. However, scientists could not agree on whether these vaccines actually worked. Different vaccines were made in different ways, and there was no standard system for testing their effectiveness.[6]

Public Health and International Response

Since doctors had no reliable cure, governments depended on public health measures to slow the disease. These included isolation, quarantine, improved hygiene, masks, disinfecting public areas, and restricting large gatherings. Different cities responded in different ways. New York City required influenza cases to be reported and required infected people to isolate either at home or in hospitals. Chicago closed theaters and other gathering places, while San Francisco encouraged or required masks in many public settings. Some activities were even moved outside because officials believed crowded indoor spaces increased the risk of transmission. These restrictions also created problems. Closing businesses and public spaces could hurt the economy, while restricting transportation could interfere with the war. Governments had to decide whether

[5] National Research Council. “A Theory of Germs.” *Science, Medicine, and Animals.*
[6] John M. Eyler. “The State of Science, Microbiology, and Vaccines Circa 1918.” *Public Health Reports.*

reducing infections was worth the economic and military consequences. The shortage of healthcare workers made the crisis worse. Many doctors and nurses were already serving in military hospitals in the United States or Europe. During the fall of 1918, the United States faced a serious shortage of professional nurses. The Red Cross recruited volunteers to care for patients, deliver supplies, prepare food, and assist overwhelmed hospitals.[7] The international response was even more difficult because the United Nations did not exist in 1918. Countries had to rely on their own governments, public health departments, military authorities, diplomatic relationships, and organizations such as the Red Cross. The Pan American Sanitary Bureau also existed as an early form of international public health cooperation. Many of the countries facing influenza were also fighting one another. Allied and Central Powers were dealing with the same disease while still being enemies. This created difficult questions over whether countries should share medical information, coordinate quarantine rules, or help enemy populations during the outbreak.[8]

Global Effects

The pandemic also affected economies and populations around the world. When large numbers of workers became sick, factories, transportation systems, farms, hospitals, and businesses struggled to operate. This was especially serious during wartime because countries depended on factories to produce military supplies and on transportation networks to move soldiers and equipment. In October 1918, New York City reported a 40 percent decline in shipyard productivity because of influenza illnesses. This shows how the disease could damage both the civilian economy and the war effort. Governments therefore faced another difficult choice: closing workplaces could slow the disease, but keeping them open could cause even more workers to become sick. The age of many victims made the crisis even more serious. Unlike most influenza outbreaks, the 1918 pandemic caused unusually high death rates among adults between about 20 and 40 years old. This group included soldiers, workers, nurses, parents, and others who were important to both the economy and the war effort. The pandemic was also affecting areas outside Europe and the United States. Developing regions and colonial territories often had fewer hospitals, doctors, and medical supplies. Poorer communities could also face crowded living conditions and limited access to healthcare, making it more difficult to isolate sick people and control outbreaks.[9] Colonial governments therefore had to decide how limited medical resources should be divided between soldiers, European populations, and people living in colonies throughout Africa and Asia.

The Crisis

It is now October 1918. The deadliest wave of influenza seen so far is spreading rapidly. Hospitals are becoming overcrowded, cities are closing public spaces, and there are not enough

[7] Centers for Disease Control and Prevention. “1918 Pandemic Influenza Historic Timeline.
[8] Pan American Health Organization. “Purple Death: The Great Flu of 1918.
[9] The 1918 Influenza Pandemic and Its Legacy.

trained doctors and nurses to care for everyone. Scientists are experimenting with vaccines, but there is still no proven cure and no agreement on the actual cause of the disease. At the same time, World War I continues. Armies are still fighting, governments are still moving soldiers across countries and oceans, and countries remain suspicious of one another. Delegates must decide how far governments should go to control the outbreak. Strict quarantines and closures could slow the disease but hurt economies and military operations. Continuing normal activity could help the war effort but spread influenza even further. Countries must also decide whether scientific information and medical supplies should be shared internationally, even between enemy nations. The actions taken during this crisis could affect not only how many people survive the pandemic, but also the final months of World War I and the world that follows it.

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